Ebola Bundibugyo virus disease, Democratic Republic of the Congo

Date and version of current assessment: 14 August 2026, v4
Date(s) and version(s) of previous assessment(s): 15 May 2026, V1; 22 May 2026,V2; 06 June 2026, V3

Overall risk and confidence[1]

Overall risk			
Democratic Republic of the Congo	**Countries sharing land borders with the Democratic Republic of the Congo**	**Rest of the African Region**	**Global**
Very High	**High**	**Low**	**Low**

Confidence in available information			
Democratic Republic of the Congo	**Countries sharing land borders with the Democratic Republic of the Congo**	**Rest of the African Region**	**Global**
Moderate	Moderate	Moderate	High

Risk statement

Since first detected in May 2026, the Bundibugyo virus disease (BVD) outbreak has rapidly evolved into a large and geographically expanding epidemic in the Democratic Republic of the Congo, with sustained transmission, high mortality and an increasing risk of further international spread. The current outbreak is the second documented Bundibugyo virus disease outbreak in the country, after the 2012 outbreak, the largest Ebola disease outbreak ever recorded in the Democratic Republic of the Congo, irrespective of Ebola virus species.

Following laboratory confirmation of Bundibugyo virus on 15 May 2026, with 13 laboratory-confirmed cases from 20 tested specimens, retrospective investigations conducted by the provincial field team identified 246 suspected cases and 65 deaths (CFR: 26.4%) across three health zones in Ituri (Mongbwalu, Rwampara and Bunia) occurring between 15 April and 13 May 2026.

By 13 August 2026, the outbreak had spread to six of the country's 26 provinces (Bas-Uélé, Haut-Uélé, Ituri, North Kivu, South Kivu and Tshopo) encompassing 54 health zones, with 4566 laboratory-confirmed cases and 2128 deaths (see map in the Annex). Since the beginning of the outbreak, 155 healthcare workers have been infected. Positively, 918 patients have recovered and been discharged from treatment centres. These figures demonstrate a substantial increase in the scale and geographic extent of the outbreak over the past three months.

Ituri Province remains the epicentre of the outbreak, accounting for 3912 confirmed cases (86%) and 1701 deaths (80%) as of 13 August. The outbreak was initially detected as a cluster in Mongbwalu Health Zone, a rural gold-mining area characterised by high population mobility linked to mining, trade and movement between communities. Spread to additional health zones within Ituri and subsequently to other provinces occurred with population movement and connections between affected communities. Healthcare facilities also contributed to the establishment of new transmission chains, with Bas-Uélé being identified as the most recently affected province on 12 August. This geographic spread has increased the complexity of the response, requiring sustained surveillance, contact tracing and infection prevention and control measures across an increasing number of affected areas. Ituri Province also has strong cross-border connectivity with Uganda and South Sudan, highlighting the continued risk of exportation.

The crude case fatality ratio (CFR) to date is 47%, underscoring the severity of the outbreak and ongoing challenges related to timely case detection, access to and quality of clinical care, and effective interruption of viral transmission. Delays in recognising cases increase the likelihood of onward transmission within households, communities and healthcare facilities.

Contact tracing and follow-up have increased significantly since the start of the outbreak, but documentation and registration remain incomplete, limiting data accuracy. As of 13 August, 18811 contacts had been identified and documented in a line list with detailed contact information, while the number of contacts reported as being under follow-up is significantly higher, but many contacts lack detailed information. This limits the ability to fully assess

[1] Confidence refers to the level of confidence in the data/information or the quality of the evidence available at the time the RRA is conducted. Poor quality information may increase the overall perceived risk due to the incertitude in the assessment.

epidemiological links, exposure histories and transmission patterns among all contacts under follow-up and indicate gaps in contact investigation and documentation.

The outbreak has also demonstrated a clear potential for cross-border spread. The first international spread was identified in Uganda in May 2026, when a symptomatic patient from the Democratic Republic of the Congo travelled to Kampala and was subsequently confirmed with Bundibugyo virus disease. Additional cases were subsequently identified in Uganda, demonstrating that population movement across the border can result in onward local transmission.

In response to the expanding outbreak and increasing risk of international spread, on the 17 May 2026, the WHO Director-General determined the outbreak in the Democratic Republic of the Congo and Uganda a public health emergency of international concern (PHEIC). On 22 May, an International Health Regulation (IHR) Emergency Committee was convened. Considering the advice of the IHR Emergency Committee, The Director-General of WHO issued temporary recommendations to all States Parties, including to strengthen surveillance, preparedness and response in countries with documented BVD, and those sharing land borders with affected countriesat highest risk of importation.

Since the start of the outbreak and as of 13 August, cases have been detected and/or treated outside of the Democratic Republic of Congo. Uganda reported 20 confirmed BVD cases during the outbreak (from 15 May 2026). The last imported case was reported on 21 June and discharged on 16 July; the 42-day enhanced monitoring period, as per international guidance, will therefore finish on 27 August. All 836 identified contacts completed the required 21-day follow-up period; during this period, six contacts, including four healthcare workers, developed BVD and were treated.

France reported a single imported BVD case on 24 June 2026, with no secondary transmission. The case was discharged on 4 July, and all five identified flight contacts completed the required 21-day follow-up period without developing symptoms. The 42-day enhanced monitoring period comes to an end on 15 August.

Germany reported two BVD cases diagnosed in the Democratic Republic of the Congo and subsequently medically evacuated to Germany for treatment. Both patients recovered and were discharged on 6 June and 28 July, respectively. The cases were managed under strict infection prevention and control measures, and no secondary transmission was reported, therefore, no contacts were identified.

Although these events demonstrate that imported cases can be detected and transmission interrupted, the continuing intensity of transmission in the Democratic Republic of the Congo means that the risk of further exportation remains. Entry and exit health screening and surveillance measures are in place at airports, ports and official land border crossings; however, movement through informal border crossing routes may occur, presenting an ongoing risk of virus exportation, importation and onward transmission.

Countries sharing land borders with the Democratic Republic of the Congo remain at risk because of frequent cross-border population movement. Uganda, the Central African Republic and South Sudan are of particular concern for importation given their proximity, high population mobility and connectivity with areas of the Democratic Republic of the Congo currently experiencing intense transmission. For the Central African Republic and South Sudan, these risks are further compounded by high humanitarian needs, population displacement, insecurity and underlying limitations in health-system capacity.

The risk in the Democratic Republic of the Congo remains assessed as very high, reflecting the current intensity and breadth of the outbreak, sustained transmission across multiple provinces and health zones, and the continued presence of epidemiological and operational factors that facilitate further transmission. The main considerations supporting this assessment are:

- **The outbreak has increased substantially since the previous risk assessment.** As of 6 June 2026, 515 confirmed cases and 91 confirmed deaths had been reported in the Democratic Republic of the Congo. By 13 August 2026, the number of cumulative confirmed cases had increased nearly nine-fold, from 515 to 4566, while the number of cumulative deaths had increased more than twenty-three-fold, from 91 to 2128. The CFR increased from 18% on 6 June to 47% on 13 August. Although the CFR reported on 6 June was noted at the time to likely be an underestimate of the true fatality ratio, the substantial increase in the CFR nevertheless highlights the severity of the outbreak and continued challenges in timely detection, referral and clinical management of cases.
- **The reported CFR may increase further as additional deaths are investigated and outcomes are established**
  More than 100 probable deaths that occurred before the outbreak was declared have been investigated and are awaiting Ministry of Health (MoH) validation; this inclusion could increase the reported CFR.
  Additionally, as of 10 August, more than 700 confirmed cases had no final outcome, excluding reported deaths, recoveries, and patients in designated isolation centres. This incomplete outcome ascertainment should be considered when interpreting the current CFR.
- **The geographic extent of the outbreak has expanded considerably since the previous risk assessment.** The number of affected health zones has increased from 25 to 54 (116%) with transmission now reported across five provinces, compared with three provinces on 6 June.
- **Ituri has the highest number of affected health zones,** (28/36), followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23), South Kivu (1/34), and Bas-Uélé (1/11), confirming the continued geographic spread of the disease. Furthermore, 45 of the 54 affected health zones have reported confirmed cases within the past 21 days, indicating ongoing active transmission across a large geographic area.

- **Ituri remains the principal focus of transmission and shows substantial evidence of ongoing, undetected transmission**. The province, which borders both Uganda and South Sudan, accounts for 86% (3912/ 4566) confirmed cases and 80% (1701/ 2128) reported deaths in the Democratic Republic of Congo. Transmission continues in both densely populated urban areas and rural settings. More than 80% of new infections in the province are detected outside known contact lists, indicating that many transmission chains remain unidentified, while retrospective investigation indicates that approximately 40% of new cases have a known epidemiological link to a previous case. Approximately two-thirds of deaths occur outside designated Ebola Treatment Centres (ETCs), suggesting delayed healthcare-seeking, late detection and continued community transmission. Although safe and dignified burial (SDB) teams are being scaled up, gaps remain in alert management, notification and investigation of deaths, systematic swabbing, contact identification and tracing, decontamination and timely implementation of SDB measures. The continued intensity of transmission in Ituri, together with its proximity to international borders and spread to additional provinces, increases the potential for further transmission within the Democratic Republic of the Congo and across borders.
- **Transmission among healthcare workers and capacity constraints in health-care settings remain a concern.** The number of confirmed infections among healthcare workers increased nearly tenfold since 6 June, from 16 to 155, including 45 deaths. This continued occurrence of infections highlights possible occupational exposure risks and gaps in infection prevention and control (IPC) implementation in healthcare facilities, which may contribute to further transmission. However, good-quality data are not currently available to determine whether exposures occurred during healthcare duties or in community settings, as only eight of the 54 affected health zones have received training to conduct surveillance activities, including detailed case investigations. At the same time, limited health-care infrastructure, insufficient Ebola treatment and isolation capacity, and inadequate ambulance availability constrain timely isolation, referral and clinical management of suspected and confirmed cases. These gaps may increase the risk of health-care-associated transmission and delayed access to care, while contributing to frustration among affected families, undermining community confidence in the response and potentially delaying care-seeking.
- **Contact tracing and follow-up have increased substantially but available documentation remains incomplete.** Although contact tracing activities have expanded substantially, more than 80% of newly reported infections continue to be detected outside known contact lists, indicating that many transmission chains remain unidentified. As of 12 August, 18811 contacts had been identified and documented with detailed information. However, the number reported as under follow-up is considerably higher, with complete line-list data not yet available for all contacts. This limits assessment of epidemiological links, exposure histories and transmission patterns. The scale of contact tracing needed, insufficient human resources, ongoing strikes among MoH responders and community health workers, and persistent pockets of community mistrust and population movement continue to challenge timely and complete contact tracing**.**
- **Ongoing conflict and insecurity in Ituri and North Kivu provinces continue to constrain response operations.** Insecurity restricts the movement of surveillance and Rapid Response Teams, limits the secure transport of laboratory specimens, and hinders contact tracing, SDB activities and community engagement. These access constraints may delay detection and investigation of cases and deaths and limit the timely implementation of response measures. In addition, insecurity may discourage individuals from seeking healthcare.
- **Laboratory capacity and testing supply constraints continue to affect timely confirmation and response.** Delays in sample transportation and, in some locations, communication of laboratory test results can delay confirmation, affect timely isolation, clinical management, contact identification and implementation of other public health measures, while also contributing to community frustration.
- **No licensed vaccine or specific antiviral treatment is currently available for Bundibugyo virus disease.** Although a randomized clinical trial for Ervebo is currently being initiated as well as the PARTNERS trial for effective treatments, the response currently relies on community engagement and early detection and isolation of all cases, intensive supportive clinical care, infection prevention and control, contact tracing, safe and dignified burials, and other public health and social measures, placing substantial operational demands on the response.
- **Community protection and engagement capacities have been strengthened but remain insufficient relative to the scale of the outbreak.** More targeted and in-depth engagement of local leadership, trusted local networks, training of community health workers (CHWs) and establishment of community brigades in high-transmission areas have strengthened community-level response capacity. Efforts to provide at-risk communities with timely and accurate information have also increased. However, available resources and capacity remain inadequate relative to the increasing scale of the outbreak compounded by persistent community mistrust and delays in scaling up essential response services, contributing to delays in referral and care-seeking, underreporting and reduced uptake of response measures.
- **Funding gaps threaten the continuity and scale of the response.** Insufficient and unpredictable funding limit the ability to sustain essential surveillance, laboratory, clinical care, infection prevention and control, contact tracing, community engagement and other response activities, particularly in areas affected by insecurity and limited access.
- **The potential for national spread remains significant.** The outbreak has expanded across six provinces and 54 health zones, including Kisangani, a major port city on the Congo River and a key link to the capital, Kinshasa. Sustained transmission, extensive population movement and major transport and trade routes linking affected and unaffected areas increase the likelihood of further geographic expansion within the Democratic Republic of the Congo.

The risk for countries sharing land borders with the Democratic Republic of the Congo remains assessed as high, reflecting the ongoing transmission and geographic expansion of the outbreak, and particularly for countries with

sustained cross-border population movement and close social and economic links with affected areas. The key factors supporting this assessment include:

- **High population mobility across formal and informal routes.** Cross-border movement associated with trade, mining, pastoral activities, family visits, seeking health care, displacement and insecurity remain frequent across the region. Movement through both official and informal crossing points, particularly between border communities and affected areas, creates opportunities for infected individuals to cross borders before detection**.**
- **Risk of undetected importation and onward transmission.** The high proportion of infections identified outside known contact lists indicates that transmission chains remain undetected in affected communities. Cases or contacts crossing international borders or being lost to follow-up may therefore result in delayed detection and onward transmission in neighbouring countries. Cross-border movement to access health services may be particularly relevant where health-care capacity is limited in affected areas of the Democratic Republic of the Congo.
- **Variable surveillance, preparedness and response capacities.** Differences in BVD surveillance and case detection, sample transportation and laboratory capacity, clinical management, infection prevention and control, contact tracing and outbreak response capabilities across neighbouring countries may affect their ability to rapidly identify and contain imported cases.
- **Operational, humanitarian and access constraints may hinder preparedness and response.** Insecurity, population displacement, limited access to health services and challenging operating environments in border areas may constrain surveillance, contact tracing, laboratory investigation, community engagements, and other preparedness and response activities in neighbouring countries.
- **Gaps in cross-border information sharing and community preparedness may delay detection and response.** Delays in sharing information on cases and contacts who cross borders, including through IHR mechanisms and direct coordination between WHO and partner response teams, may hinder timely follow-up. Limited resources and uneven capacities to train, equip and support CHWs for community-based surveillance and RCCE, generate and use timely community evidence and engage trusted local leaders and community networks may further delay care-seeking and detection and constrain rapid response following an imported case.

The risk for the rest of the African Region and at the global level remains assessed as low, based on the available epidemiological evidence and the absence of widespread or sustained transmission beyond the main affected areas. The key considerations supporting this assessment include:

- **Transmission remains concentrated in the Democratic Republic of the Congo.** The majority of reported cases and deaths remain concentrated in the Democratic Republic of the Congo, however, transmission has also been documented in Uganda, and a travel-associated case was detected in France. These events demonstrate that the virus can cross international borders through population movement and underscore the importance of strengthened surveillance, early detection, laboratory capacity, infection prevention and control, and response readiness in countries with epidemiological and population-mobility links to affected areas. Accordingly, regional and global preparedness has been increased.
- **There is currently no evidence of sustained transmission beyond the Democratic Republic of the Congo.** The continued outbreak in the Democratic Republic of the Congo presents a risk of further exportation, particularly to countries with strong population and travel links, but available evidence does not indicate ongoing international transmission.
- **International exportation remains possible**. Individuals infected in the Democratic Republic of the Congo may travel during the incubation period before symptoms develop, and cases could therefore be detected in other countries. However, in the absence of evidence of sustained transmission outside the affected areas both in this outbreak and historically in previous Ebola outbreaks, this possibility does not currently warrant an increase in the overall regional or global risk assessment. Despite the risk of wider regional and global spread remaining limited, continued vigilance is required for surveillance, rapid detection and investigation of suspected cases, and appropriate preparedness in countries with travel and population links to the Democratic Republic of the Congo to ensure that any exported cases are promptly identified and contained.

Risk questions

Risk question		Assessment		Risk	Rationale
		Likelihood	Consequences		
Potential risk for human health?	Democratic Republic of the Congo	Highly likely	Major	Very high	The risk to human health in the Democratic Republic of the Congo is assessed as very high, given the evidence of sustained transmission,

	Countries sharing land borders with Democratic Republic of the Congo	Likely	Moderate	High	rapid increases in the number of cases and deaths, as well as in the number of infections occurring among healthcare workers. The highest proportion of deaths is reported among children below ten years of age. Cases have also been reported among vulnerable populations, including in camps for internally displaced persons located in Ituri. Based on analysis of 139 genomic sequences from BDBV samples collected between May and June 2026, it is estimated that transmission began several months before the outbreak was detected, resulting in more undiagnosed infections and exposed contacts. Ongoing delays in detection, challenges in infection prevention and control, and difficulties in ensuring safe and dignified burials may further increase the likelihood of human-to-human transmission. The absence of a licensed vaccine and specific therapeutics for Bundibugyo virus also increases the potential health impact of infection. The risk to human health in neighbouring countries sharing land borders is considered high, as evidenced by imported infection followed by local transmission in Uganda. Continued population movement from affected areas creates opportunities for additional infections, while delayed recognition of imported cases could allow onward transmission before appropriate isolation and control measures are implemented. For the remainder of the African Region and globally, the risk to human health is assessed as low. Although imported cases cannot be excluded, there is currently no evidence of sustained transmission outside the affected areas.
	Rest of African Region	Unlikely	Minor	Low	
	Global	Unlikely	Minor	Low	
Risk of event spreading?	Democratic Republic of the Congo	Highly likely	Major	Very high	The risk of further spread within the Democratic Republic of the Congo remains very high. Since the outbreak was first detected in May 2026, transmission has spread to 54 health zones across six provinces; of which 45 health zones have reported confirmed cases during the preceding 21 days, indicating that transmission remains active across a wide geographical area. More than 80% of new infections are being identified outside known contact lists, and 60% remain unlinked even after investigation, indicating that a substantial proportion of transmission chains continue to go undetected,increasing the likelihood of continued spread. The potential for further geographic expansion is reinforced by the concentration of transmission in Ituri, which accounts for 85.7% of confirmed cases, while transmission has also been reported in North Kivu, South Kivu, Haut-Uele and Tshopo. High population mobility, including movement related to trade, mining, displacement and insecurity, provides additional opportunities for transmission between affected and unaffected areas. Operational constraints, including insecurity, limited access, difficulties with contact tracing and laboratory capacity, and insufficient isolation and treatment capacity, may further delay identifying and interrupting transmission chains.
	Countries sharing land borders with Democratic Republic of the Congo	Likely	Moderate	High	
	Rest of African Region	Unlikely	Minor	Low	
	Global	Unlikely	Minimal	Low	

					The risk of further spread to neighbouring countries sharing land borders is assessed as high. The geographical expansion of the outbreak, including in provinces bordering other countries, combined with substantial cross-border population movement through formal and informal routes, creates continued opportunities for exportation. Undetected transmission chains in the Democratic Republic of the Congo further increase the likelihood that infected individuals may cross borders before detection. Differences in surveillance, laboratory, contact-tracing, and response capacities among neighbouring countries could also affect how quickly imported cases are identified and contained. Uganda, the Central African Republic and South Sudan are of particular concern for (re-)importation, given their geographic proximity (direct border), high population mobility and cross-border connectivity with areas of the Democratic Republic of the Congo currently experiencing intense transmission. The risk of wider spread across the African Region and globally remains low. Although international exportation during the incubation period remains possible, there is currently no evidence of sustained transmission outside the Democratic Republic of the Congo. Previous exported cases have not resulted in sustained international transmission, supporting the current low assessment. Continued surveillance and rapid investigation of suspected imported cases remain important to prevent isolated introductions from developing into onward transmission.
Risk of insufficient control capacities with available resources	Democratic Republic of the Congo	Highly likely	Major	Very high	The risk associated with insufficient outbreak control capacity in the Democratic Republic of the Congo is assessed as very high. The current response is being implemented amid substantial operational constraints, including shortages of PPE, challenges in contact tracing, delays in accessing laboratory confirmation and results, insufficient healthcare and isolation capacity, difficulties maintaining surveillance and response activities in affected areas, and underfunding. Funding gaps and unpredictable resources may also affect the continuity and scale of essential surveillance, laboratory, clinical, IPC, contact-tracing and community-engagement activities. Contact tracing is ongoing, but registration and reporting remain incomplete, limiting data accuracy.

	Countries sharing land borders with Democratic Republic of the Congo	Likely	Minor	High	The high proportion of infections detected outside known contact lists indicates gaps in contact tracing, case finding and the capacity to identify and interrupt transmission chains promptly. Response capacity is further constrained by insecurity, population displacement and restricted access to affected areas, particularly in Ituri and North Kivu. These conditions can limit the deployment of surveillance and rapid response teams, hinder specimen transport and laboratory investigation, and complicate contact tracing and safe and dignified burials. Limited healthcare infrastructure and insufficient isolation and treatment capacity may also compromise timely case management and infection prevention and control. In addition, shortages of trained healthcare workers and competing health emergencies, including malaria, cholera and mpox, place additional pressure on an already constrained health system. Health workers involved in the Bundibugyo virus response have undertaken strikes in affected areas in response to delays in salary and outbreak-response allowance payments, alongside difficult working conditions and significant occupational and logistical challenges. These strikes have disrupted treatment and response activities and may further constrain clinical care, surveillance, contact tracing, community engagement and safe burial activities. Community-level challenges may further reduce the effectiveness of available response resources. Gaps in community engagement, social listening, rumour management and trust in response activities may contribute to delayed care-seeking, underreporting and reduced acceptance of public health interventions. For countries sharing land borders with the Democratic Republic of the Congo, the risk associated with insufficient outbreak control capacity is assessed as high. Countries in the region have strengthened preparedness and readiness measures, including enhanced surveillance at points of entry (PoEs) and activation of contingency plans. However, continued population movement across porous borders requires sustained surveillance and rapid response capacity. Differences in laboratory, healthcare, surveillance, and outbreak-response capacities may delay detection, investigation, and containment of imported cases, particularly in settings with limited resources. At the regional and global levels, the risk associated with insufficient outbreak control capacity remains low, as there is currently no evidence of sustained transmission beyond the Democratic Republic of the Congo requiring large-scale international response capacities. Nevertheless, continued preparedness and the availability of resources for rapid investigation and containment of any exported cases remain important.
	Rest of African Region	Likely	Moderate	Moderate	
	Global	Unlikely	Minimal	Low	

Major actions recommended by the risk assessment team

	Action	Timeframe
☒	Refer the event for review by IHR Emergency Committee for consideration as a PHEIC by DG (Art 12, IHR)	Immediate

☐	Immediate activation of ERF response mechanism (IMS) as urgent public health response is required	Not applicable
☐	Recommend setting up of grading call (funding can be accessed before grading completed)	Not applicable
☐	Immediate support to response, but within limit of CFE (no grading recommended at this point in time)	Not applicable
☐	Rapidly seek further information and repeat RRA (including field risk assessment)	Not applicable
☒	Support Member State to undertake preparedness measures	Continuous
☒	Continue to closely monitor	Continuous
☐	No further risk assessment required for this event, return to routine activities	Choose an item.

Immediate actions

- Substantially scale up the deployment of multidisciplinary surge support teams to the newly affected Bas-Uélé province, as well as to Mongbwalu, Rwampara, Bunia, Lita, Nizi, and other affected health zones across Ituri and other affected provinces
- Initiate and maintain comprehensive epidemiological investigation (including support of genomic sequencing where feasible and if needed) to clarify chains of transmission and guide timely interventions.
- Implement robust contact identification, exhaustive contact listing and consistent follow-up in all affected areas.
- Strengthen surveillance and alert management (including community-based surveillance and screening at all PoEs), including in neighboring countries not currently affected but at high risk for importation.
- Improve digitization of alerts, case line listing, contact listing/tracing, and laboratory data to support timely data flows for decision making by response teams.
- Strengthen community protection as a core response pillar, by scaling up RCCE, social listening, community feedback systems, infodemic management, community-based surveillance, safe and dignified burial engagement, and CHW mobilization.
- Establish and scale-up RCCE activities within affected and at-risk communities, including engagement of local leaders, faith-based organizations, civil society organizations, internally displaced populations, mining communities, mobile populations, and cross-border communities.
- Strengthen the collection, analysis and operational use of community insights through social listening, community feedback mechanisms and community-based surveillance. Integrate these into Emergency Operations Centre (EOC) and Incident Management System (IMS) decision-making processes.
- Reinforce IPC standards and triage in health facilities in affected and high-risk areas, including isolation capacities, safe and dignified burials, and PPE for health workers, in addition to promotion of basic IPC practices at the community level.
- Support rapid establishment and/or scaling of Ebola treatment units and isolation facilities in priority locations, ensuring referral pathways and safe transport.
- Preposition and distribute essential supplies (PPE, sample collection materials, reagents, essential medicines, WASH kits, safe-burial kits) to strategic locations and healthcare facilities in affected and at-risk areas.
- Expand access to BVD diagnostic testing by continuing the decentralization and strengthening BDBV RT-PCR capacity, ensuring adequate and sustained supply of compatible reagents and consumables, functional and rapid sample transport, and timely reporting from national and sub-national laboratories.
- Scale up readiness support in neighbouring countries, including community protection, RCCE, CHW preparedness, community-based surveillance, social listening, cross-border engagement and readiness training packages.

Supporting information

Hazard assessment

Bundibugyo virus disease (BVD), a form of Ebola disease, is a severe viral haemorrhagic disease caused by Bundibugyo virus (BDBV), a member of the *Orthoebolavirus* genus in the *Filoviridae* family. Six virus species are currently recognized within the genus, including Ebola virus, Sudan virus and Bundibugyo virus, which have caused major Ebola disease outbreaks in Africa. BDBV was first identified during an outbreak in western Uganda in 2007, followed by a second documented outbreak in the Democratic Republic of the Congo in 2012. The current event is the third documented BDBV outbreak. The previous BVD outbreaks reported CFR of approximately 30%

BDBV is considered a zoonotic pathogen, although its natural reservoir has not been conclusively established. Several bat species are considered possible natural hosts of orthoebolaviruses, while non-human primates, including monkeys, gorillas and chimpanzees, can develop severe disease following infection. Human infection may occur through contact with the blood or other body fluids of infected animals, followed by human-to-human transmission through direct contact with the blood or body fluids of infected persons, or with contaminated materials and surfaces. Contact with the bodies of deceased infected persons can also result in transmission, particularly during unsafe burial practices. For other filoviruses, rare instances of transmission have been linked to viral persistence in body fluids of survivors (e.g. semen). There is currently no data on Bundibugyo virus persistence in recovered individuals, but similar persistence is likely.

The incubation period ranges from 2 to 21 days**.** BVD typically begins with non-specific symptoms including fever, fatigue, muscle aches, headache and sore throat. These may be followed by vomiting, diarrhoea and rash, with severe disease potentially involving impaired kidney and liver function and internal or external bleeding. The non-specific nature of the initial symptoms may make early clinical recognition difficult, particularly where other febrile infectious diseases are circulating.

Laboratory confirmation is required to verify BDBV infection. Diagnosis relies primarily on molecular methods, including reverse transcription polymerase chain reaction (RT-PCR) assays to detect viral RNA. There are currently no validated rapid diagnostic tests available for BDBV infection. Timely access to appropriate laboratory capacity is therefore important to differentiate BDBV infection from other causes of viral haemorrhagic fever and other infections.

Currently, there is no licensed vaccine for the prevention of BVD and no specific treatment approved for BDBV infection. However, BDBV-specific vaccine candidates are under development, including rVSV-Bundibugyo and ChAdOx1-Bundibugyo, which are being prioritized for clinical evaluation. Clinical management is thus supportive, and early access to appropriate medical care can improve the likelihood of survival. The absence of licensed preventive and specific therapeutic options increases the potential public health impact of an outbreak.

The combination of efficient human-to-human transmission through contact with infected body fluids, a relatively long incubation period of up to 21 days, non-specific early symptoms, a potentially severe clinical disease, limited availability of specific medical countermeasures and the absence of a conclusively identified animal reservoir contributes to the public health significance of BDBV.

Exposure assessment

Since the initial detection, the size of the population with potential exposure to BDBV has increased substantially as the outbreak has expanded geographically and transmission has remained active. Exposure is currently highest in affected communities in the Democratic Republic of the Congo, particularly in Ituri Province, which accounts for the majority of reported cases. The population at greatest risk of exposure is concentrated in communities living in and moving through areas with active transmission.

Exposure is of particularly concern among household members and other close contacts of cases, caregivers, people providing care outside formal health facilities, and persons involved in the preparation and burial of

deceased individuals. Exposure risk may also be increased in settings affected by population displacement, overcrowding and limited access to healthcare and essential services: the outbreak is occurring in a complex humanitarian and conflict-affected environment, with limited access to healthcare, water, food, shelter and protection, including overcrowded sites for internally displaced persons. These conditions may increase opportunities for close contact and make timely identification and isolation of suspected cases more difficult.

Healthcare worker infections remained a major concern, with 155 infections, including 45 deaths, reported among frontline healthcare workers since the start of the outbreak. Healthcare workers represent approximately 3.5% of all confirmed cases. Infections among healthcare workers indicate ongoing occupational exposure and challenges in maintaining adequate infection prevention and control measures in healthcare facilities. Exposure may occur through direct patient care, contact with body fluids or contaminated materials, and during procedures or management of patients before BDBV is recognized. Healthcare workers may also be exposed as members of affected communities.

The number of persons requiring contact monitoring has also increased substantially as the outbreak has expanded. As of 13 August, 18811 contacts had been identified and documented in a line list with detailed contact information, while the number of contacts reported as being under follow-up is significantly higher but many contacts lack detailed information. The large number of contacts reflects the scale of potential exposure within affected communities. Insecurity, attacks on health facilities, population displacement, and restricted access to affected areas may limit response teams' ability to identify and follow all potentially exposed individuals, creating the possibility of continued exposure before cases are detected. Furthermore, a lack of training among a large proportion of case investigators and contact tracers may lead to inadequate contact listing and follow-up, respectively.

Population mobility represents an additional pathway for exposure and spread. Movement associated with mining, trade, displacement, family visits, pastoral activities, seeking health care and other routine travel between communities and across international borders may allow infected individuals to move between affected and unaffected areas before detection and diagnosis. Movement through both formal and informal crossing points may facilitate the introduction of infection into neighbouring communities and countries.

Exposure outside the Democratic Republic of the Congo remains possible through undetected exportation, particularly to neighbouring countries with established population and cross-border movement links. Uganda, which shares a border with affected areas of the Democratic Republic of the Congo, previously reported imported BDBV cases, with subsequent local transmission. The last imported case was discharged on 16 July 2026. Nevertheless, Uganda and other neighbouring countries remain at risk of imported cases and (re-)introduction because transmission is ongoing in bordering areas of the Democratic Republic of the Congo and population movement continues.

At the international level, exposure through infected travellers remains possible. However, because individuals are not considered infectious during the 2–21-day incubation period before symptoms develop, travel that occurs during this window does not put others at risk. While an infected traveller was identified in France, this incidence is isolated and there is currently no evidence of sustained transmission outside the Democratic Republic of the Congo. Therefore the likelihood of significant ongoing exposure in other countries remains low.

Context assessment

The current outbreak represents an unprecedented event for Ebola disease, in terms of both the number of cases reported in a single country over a short period and the speed of its expansion. It is now substantially larger than both of the previous BVD outbreaks. Within approximately three months, 4566 confirmed cases have been reported, already exceeding the total number of cases recorded during the 2018–2020 Ebola virus disease outbreak in the Democratic Republic of the Congo, which lasted almost two years.

Comparison of current outbreak to the previous BVD outbreaks and the largest Ebola disease outbreaks

Outbreak	Virus	Cases	Deaths and (crude CFR)	Approx. duration

BVD, Democratic Republic of the Congo, 2026	Bundibugyo virus	4,566 confirmed	2128 (47%)	~3 months so far
BVD, Uganda, 2007–2008	Bundibugyo virus	149 total (131 confirmed and 18 suspected)	42 (28%) (37 confirmed and 5 suspected)	~5–6 months
BVD, Democratic Republic of the Congo, 2012	Bundibugyo virus	56 confirmed	17 (30%)	~3 months
SVD, Uganda, 2000–2001	Sudan virus	425 confirmed	224 (53%)	~5 months
SVD, Uganda, 2022–2023	Sudan virus	164 total (142 confirmed, 22 probable)	77 (47%)	~2 months
EVD, Democratic Republic of the Congo, 2018–2020	Ebola virus	3,481 confirmed	2 299 (66%)	~23 months
EVD, West Africa, 2014–2016	Ebola virus	More than 28,600 infected	11 325 (≈ 40%)	~27 months

The outbreak is occurring in eastern Democratic Republic of the Congo, an area with a recurrent history of Ebola disease outbreaks and strong epidemiological and population links with western Uganda and southern South Sudan. This ecological and population corridor has demonstrated vulnerability to filovirus emergence and transmission, creating a potential pathway for cross-border spread. Retrospective investigations suggest that transmission may have begun several months before the outbreak was officially recognized, with suspected viral haemorrhagic fever cases dating back to January 2026. This potential extended period of unreported transmission, together with the continued detection of cases and expansion into additional health zones, suggests that the reported figures may not yet capture the full extent of the outbreak and that substantial ongoing transmission may remain undetected.

Epidemiological expansion and acceleration

The outbreak has expanded substantially and now affects 54 of 140 health zones across six provinces: Ituri (28/36 health zones), North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23), South Kivu (1/34), and Bas-Uélé (1/11). Transmission remains active in 45 of the 54 affected health zones, defined as health zones that have reported at least one case within the past 21 days. Ituri remains the principal epicentre, accounting for almost 86% of all confirmed cases (3912) and 80% of reported deaths (1701/2128) nationally. Within Ituri, the highest numbers of confirmed cases have been reported in Bunia (27%), Rwampara (21%), Mongbwalu (15%), and Nizi (13%) health zones. Bunia and Rwampara are located relatively close to the border with Uganda, highlighting the potential for cross-border spread along this major transmission corridor.

Trend of cumulative confirmed cases and deaths, as of 11 August 2026

Confirmed Ebola Cases and Deaths - DRC

Data as of 2026-08-11

Reporting date	Confirmed cases	Increase since previous report	Approx. weekly increase	Deaths	Increase in deaths	Approx. weekly increase
25 May	106	—	—	12	—	—
08 June	598	+492	+246/week	115	+103	+ 52/week
22 June	1,094	+496	+248/week	277	+162	+ 81/week
06 July	1,708	+614	+307/week	580	+303	+152/week
20 July	2,473	+765	+382/week	999	+419	+210/week
03 August	3,874	+1,401	+700/week	1,751	+752	+376/week
11 August	4,566	+692	+606/week	2,128	+377	+330/week

Source: Centre des Opérations d'Urgence de Santé Publique (COUSP) situation reports

Please interpret the above table with caution, noting that a batch reporting seen on July 22 reflects a backlog reconciliation of 272 cases and 174 deaths, not a true weekly surge in cases.

Note: the final reporting period covers 8 days rather than a full 14-day interval, as it reflects data up to the current reporting date.

The outbreak showed a marked acceleration from late May through early August 2026, with the number of confirmed cases increasing from 106 on 25 May to 4566 by 11 August, while the number of confirmed deaths increased from 12 to 2128 (see Annex for epi curve). The estimated weekly incidence of confirmed cases rose from approximately 246 cases per week in early June to 700 cases per week by early August, before declining slightly to 606 cases per week in the most recent reporting period. Similarly, the estimated weekly increase in deaths rose from 52 deaths per week in early June to 376 deaths per week by early August, remaining high at 330 deaths per week through 11 August. Although part of this increase likely reflects expanded surveillance, enhanced laboratory testing, improved diagnostic capacity, and periodic backlog reconciliation, the sustained rise in number of cases and deaths indicates continued transmission and substantial outbreak expansion.

The acceleration was particularly pronounced between 6 July and 3 August, when the number of confirmed cases increased from 1708 to 3874 (+2,166; +127%), while the number of confirmed deaths increased from 580 to 1751 (+1,171; +202%). During the most recent reporting period, the number of cases increased further to 4566 and deaths to 2128, indicating that transmission remained intense despite ongoing response efforts. By 11 August, 918 patients had recovered.

Population mobility and regional connectivity

Population movement linked to mining, informal trade, displacement and routine travel creates pathways for transmission within the Democratic Republic of the Congo and across international borders, including through informal crossing points. Bunia is a major commercial, transport and health-care referral hub connecting affected communities with other parts of the country and neighbouring countries. Kisangani, an affected port city on the Congo River, is linked by transport and trade routes to Kinshasa. These connections increase the likelihood that infected people may travel before diagnosis and introduce the virus into new communities, including the capital.

Humanitarian and security context

The outbreak is unfolding in a conflict-affected humanitarian context marked by insecurity, armed violence and large-scale displacement. More than 26 million people face acute food insecurity, and an estimated one million internally displaced people live in Ituri Province alone. Insecurity and displacement disrupt access to health care and essential services, restrict access for response teams, and impede surveillance, case investigation and contact follow-up.

Overcrowding, limited water, sanitation and hygiene services, and restricted access to health care in mining communities, informal settlements and sites for internally displaced people further hinder early detection and infection prevention. These conditions also make it harder to implement response measures consistently and to reach affected populations.

Health-system and response capacity

Health-system and response capacity remains under severe pressure from workforce shortages, operational constraints and insecurity. Delayed payment of response personnel, uneven trust in response activities and restricted access have disrupted key interventions. Although contact follow-up has improved, it remains below the 95% operational target, limiting the rapid identification and interruption of transmission chains.

Transmission continues in communities and health-care settings. Frequent community deaths and delayed case detection indicate that some infections are identified late or remain undetected. Infections among health-care workers also point to persistent gaps in infection prevention and control.

Outbreak control is further constrained by gaps in active case finding, case investigation, contact tracing, community engagement and continuity of care. Laboratory capacity has expanded but remains under pressure as case numbers rise, while treatment and isolation services remain insufficient. The health system is also managing concurrent threats, including malaria, measles, cholera and mpox.

Security incidents affecting health facilities have restricted access and disrupted surveillance and other control activities. Funding shortfalls, logistical constraints and health-worker strikes linked to delayed remuneration have also affected contact tracing, infection prevention and control, community engagement, and safe and dignified burials. Together, these constraints delay case detection, leave transmission chains unidentified and undermine the continuity of essential response operations.

The identification of Bundibugyo virus creates additional challenges for outbreak control compared to Ebola virus outbreaks. Vaccines and therapeutics currently available for Ebola virus disease are not approved for Bundibugyo virus disease. Evidence on Bundibugyo virus disease also remains limited because only three outbreaks, including the current event, have been recognized. This constrains clinical guidance, preparedness and the selection of control measures.

Cross-border preparedness and response capacity

The risk of further cross-border transmission remains significant. Uganda previously reported 20 outbreak-related cases—15 imported and five secondary infections among contacts and health-care workers—demonstrating that an imported case can lead to local transmission. No new cases have been reported in Uganda; the last confirmed patient was discharged on 16 July 2026, and all identified contacts completed follow-up. However, continued movement through formal and informal border routes sustains the possibility of further importation, particularly to South Sudan and the Central African Republic, which have close population links with affected areas.

Preparedness and response capacity varies among countries sharing land borders with the Democratic Republic of the Congo. Countries with previous Ebola outbreak experience generally have stronger surveillance, laboratory, infection prevention and control, and response systems. Other neighbouring countries have strengthened readiness, including travel-related measures such as entry screening for travellers from the Democratic Republic of the Congo, but subnational capacity remains uneven.

In France, a responder returning from the outbreak area reported symptoms to airport authorities, was immediately isolated and was subsequently confirmed with Bundibugyo virus disease. No secondary transmission occurred. This event illustrates the value of early reporting by symptomatic travellers, rapid isolation and implementation of the Emergency Committee's temporary recommendations on border health and international travel.

Overall risk considerations

Overall, transmission across 54 of 140 health zones (38.6%), 4,566 confirmed cases and 2,128 deaths, continued infections among health-care workers, extensive contact-tracing demands, high population mobility, insecurity, displacement and restricted access make the outbreak difficult to control. These factors support a very high risk of continued spread within the Democratic Republic of the Congo.

The risk remains high for countries sharing land borders with the Democratic Republic of the Congo because population movement continues and importation has already resulted in local transmission. The risk for the rest of the African Region and globally remains low because no sustained transmission has been reported outside the affected region and mechanisms for surveillance, preparedness and response remain in place.

Capacities: Democratic Republic of the Congo	Vulnerabilities: Democratic Republic of the Congo
Leadership and coordination • Health authorities have mobilized response structures and reactivated the Ebola disease coordination committee. • On 7 June, The Inter-Agency Standing Committee (IASC) activated a System-Wide Scale-Up for the control of infectious disease events in the Democratic Republic of the Congo for an initial three-month period. • The activation aims to strengthen leadership and coordination, improve humanitarian access and advocacy, mobilize additional operational capacity, and support resource mobilization for both the Ebola response and broader humanitarian needs. ***Surveillance*** • The Democratic Republic of the Congo has experience with alert systems, active case search, case investigation and contact tracing from previous BVD and other filovirus outbreaks. • A strong digital health information system (DHIS2) was already being used even before its use in the current outbreak • Surveillance tools are available in the country • Training material is available ***Laboratory*** • The Democratic Republic of the Congo has previous experience in responding to Ebola disease, including possessing reference laboratories capable of diagnosing BDBV infection and of conducting additional investigation such as genomic sequencing. • In the Democratic Republic of the Congo, capacities have been scaled up, under the INRB leadership, with currently 19 laboratories with molecular testing capacities in six provinces: 12 laboratories in Ituri, 2 in North Kivu, 1 in South Kivu, 1 in Tshopo, 2 in Haut Uele. Testing capacities currently rely on open PCR platforms and near POC platforms. • The reported testing turnaround time is under 24h. • Decentralisation of BDBV diagnostic testing capacities continues, with new labs planned to be established in the coming weeks. • Initial data on the near POC platform and kits utilised (RadiOne) under the initial national decentralization plan are available. Additional performance data is needed to further support the decentralization strategy. ***Clinical management and IPC*** • The Democratic Republic of the Congo has good clinical experience in the management of Ebola disease outbreaks. • Training materials, evidence-based guidelines, treatment protocols, and job aids support delivery	***Coordination limitations in remote or insecure areas*** • Delays in implementing decentralized response coordination (zonal response approach) in some HZ, combined with incomplete and untimely reporting, continue to hamper local coordination and timely, context-specific decision-making. • Challenges of payment of frontline workers • Inter-community conflicts • Insufficient funding allocated • Slow feedback loops and infodemic management in remote and insecure zones • Trust remains fragile in conflict-affected areas • Community mistrust • More capacity building needed, including of African Health Volunteers Corps (AVoHC) and Strengthening and Utilizing Response Groups for Emergencies (SURGE) and Community health workers (CHWs). ***Surveillance*** • Limited human resources for active case search, case investigation and contact tracing • Late reporting/delayed detection & verification • Limited epidemiological information/ investigation • Need to strengthen cross border surveillance and information sharing. • Need for training including in health zones with no prior experience of responding to an Ebola disease outbreak • Persistent DHIS2 data synchronization issues continue to cause discrepancies between SitRep and DHIS2 figures ***Laboratory*** • BDBV testing capacity remains fragile in Democratic Republic of the Congo with risks of stock outs and limited numbers of suppliers of required equipment and reagents, and reliance on swift specimen transportation to laboratories for timely investigations. • Delays in issuance and communication of laboratory results. • There are currently no validated rapid diagnostic tests available for BDBV infection. • Challenges to scale up clinical testing capacities in support of the scale up of care capacity (equipment, supplies, human resources). ***Clinical management and IPC*** • Insufficient isolation and care capacity for the current and projected number of cases. • Lack of well-formulated screening and triage procedures. • Severely limited capability / capacity for management of critical illness. • Saturation of capacity amongst some partner organizations, limiting expansion of bed numbers.

of high-quality care and rapid expansion of healthcare worker (HCW) capacity .
- Experienced clinical and administrative teams from WHO, the Ministry of Health, and partners including ALIMA, IMC, Medair, MSF, and Samaritan's Purse are already deployed to support Ebola Treatment Centres across affected areas.
- Improvements in supply chain estimation match immediate and projected clinical needs with delivery.
- Use of standardised design principles for treatment centres enhances patient visibility, efficiency and the capability to provide high-quality care.

Risk communication and community engagement - RCCE

- The Democratic Republic of the Congo has substantial experience in Ebola-related risk communication and community engagement (RCCE)
- There are functional National RCCE and Infodemic coordination structures led by the MoHs at national and subnational levels.
- There is an operational presence of partners (WHO, IFRC, UNICEF, MSF, Alima, IFRC/Red Cross and other NGOs)
- There are existing engagement networks (CHWs, community relays, Red Cross Volunteers, religious groups, women and youths' associations)
- Rapid door-to-door mobilization capacity is available.
- Strong community dialogue experience exists and existing community channels are largely trusted (SMS campaigns, call centre, WhatsApp groups, interpersonal communication).
- RCCE materials, multilingual messages and narratives are available.
- Joint messaging has been developed with partners for affected populations and specific audiences, including displaced populations, refugees, and travellers. An Ebola social media toolkit is also available and can be translated into local languages to support RCCE activities.
- Trained staff under the joint Africa CDC and WHO emergency programe African Volunteers Health Corps (AVoHC Surge).
- Experience in integration of RCCE linked to community-based surveillance, surveillance, SDB, case management, etc.

Border health, PoEs and mass gatherings

- Exit screening at main airports, ports and ground crossings is officially in place in the Democratic Republic of the Congo.
- Population mobility mapping exercises have been conducted to inform the geospatial location of response and readiness interventions.
- Risk communication materials targeting individual travellers and small gatherings' attendees are available.
- Authorities in at-risk countries, including Uganda, South Sudan, Tanzania, Rwanda and Kenya, have been trained on the use of the WHO All-Hazards Risk Assessment Tool for Mass Gatherings.

Operational support and logistics

- Substantial scale-up of surge deployments.

- Wide geographical extent of the outbreak, which requires dynamic, rapid and decentralised scale up of clinical care to support safe treatment of patients close to the community as possible.
- Competing medical priorities due to other routine morbidities, concurrent outbreaks, and the humanitarian situation.
- Threat to continuity of care due to health and care worker infections.
- Patchy availability of safe infrastructure in which to deliver care, particularly in smaller treatment centres.
- Risk of stigma amongst people with active disease and survivors.
- Self-discharge against medical advice continues to represent a risk to patients, staff and the wider community.
- Essential care services are impacted by diversion of staff, lack of confidence amongst community to attend, and ongoing disruption to transport and health care seeking due to sporadic violence and other reasons.

Risk communication and community engagement - RCCE

- Challenges linked to complex context with incidents of community resistance, e.g., due to perceived monetization of Ebola.
- Limited funding and human resources for RCCE and infodemic management interventions.
- Need for scaling up resources to support scaling and trust building interventions.
- Partners mapping and coordination to be strengthened.
- Gaps RCCE-infodemic management capacity (resources and training).
- Community feedback mechanism (social listening, call centres, community dialogues, community data) to be strengthened among partners to track community concerns, questions and damaging narratives and to integrate into the response,
- Risk of poor use of evidence and poor triangulation of data across pillars, which indicates a need for a strengthened social sciences analytical cell, that will collect quantitative and qualitative data across pillars, triangulate, analyse and provide key recommendations.
- Community rumours, misinformation, and mistrust are hindering case detection, reporting, health-seeking behaviour and delaying safe burials.
- Security challenges continue to limit access to affected communities and impede effective community engagement.
- Marginal groups and most at-risk groups left behind.

Border health, PoEs and mass gatherings

- With BVD cases reported in Kisangani, a major port city linking the Congo River to Kinshasa, and connecting to the Republic of the Congo, and indirectly to the Central African Republic (CAR) and Angola, the potential for further geographic spread is higher. Surveillance and RCCE need to be further reinforced along this major trade and travel route.
- Many river sites are informal landing sites, small private ports or informal congregation points, the extensive use of these informal landing sites and

• WHO has deployed 264 field personnel across the Democratic Republic of the Congo and Uganda to support response operations. • Logistics corridors from Kinshasa in the Democratic Republic of the Congo, Nairobi in Kenya, Dakar in Senegal, and other regional hubs remain operational, facilitating the movement of emergency supplies and equipment to affected areas. To date, 38 metric tonnes of medical equipment, laboratory consumables, personal protective equipment (PPE), and other emergency supplies have been transported to support response operations across Ituri, North Kivu, and South Kivu provinces. WHO has also received an additional five metric tonnes of emergency supplies through support from the United Nations Organization Stabilization Mission in the Democratic Republic of the Congo (MONUSCO). • Cargo flights and logistics support continue to facilitate the delivery of critical response commodities despite persistent access challenges in some affected areas. Operational assessments of warehousing, stock positioning, and supply chain management continue to support commodity visibility and distribution across affected provinces.	small private ports presents challenges for surveillance, alert detection, referral and public response, as these locations often operate outside formal public health oversight, creating potential gaps in early warning and response systems. • Other persistent challenges at PoEs are the payment of frontline workers; security concerns in relation to compliance with voluntary screening measures, lack of equipment and supplies, lack of training for front line workers, and access to the up-to-date list of cases and contacts in a timely manner for conducting exit screening at PoEs. • The postponement of mass gatherings (MG) held within areas with documented BDBV detection needs to be considered until BVD transmission is interrupted. For mass gatherings proceeding outside areas with documented BDBV detection, a risk-based approach should be applied, with decisions resting with national or local authorities and event organizers. Despite scaling down of MGs, in such settings, the high density and mobility of attendees favour close, frequent and prolonged interactions, which increase the risk of amplification of BVD transmission and potential disruption of health services. • Small gatherings can contribute to increased disease transmission; during Ebola outbreaks, some gatherings especially funerals can increase the risk of transmission. Private and public small gatherings need particular attention, especially during outbreaks. ***Operational support and logistics*** • Transport and fuel shortages for field operations. • Stock-outs of essential supplies (PPE, IPC materials). • Lack of resources for communication costs and communication equipment. • Lack of resources for social listening (cost of software, HR). • HR: tensions due to delayed payments of salaries, refusal of mobile payment, inconsistent payment/DSA scales between partners. • Recruitment: tensions with communities due to origins/ethnical group. Localised recruitment should be prioritised.

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Annex:

Figure 1: Geographic distribution of cumulative confirmed Bundibugyo virus disease cases and affected health zones in the Democratic Republic of the Congo, as of 12 August 2026.

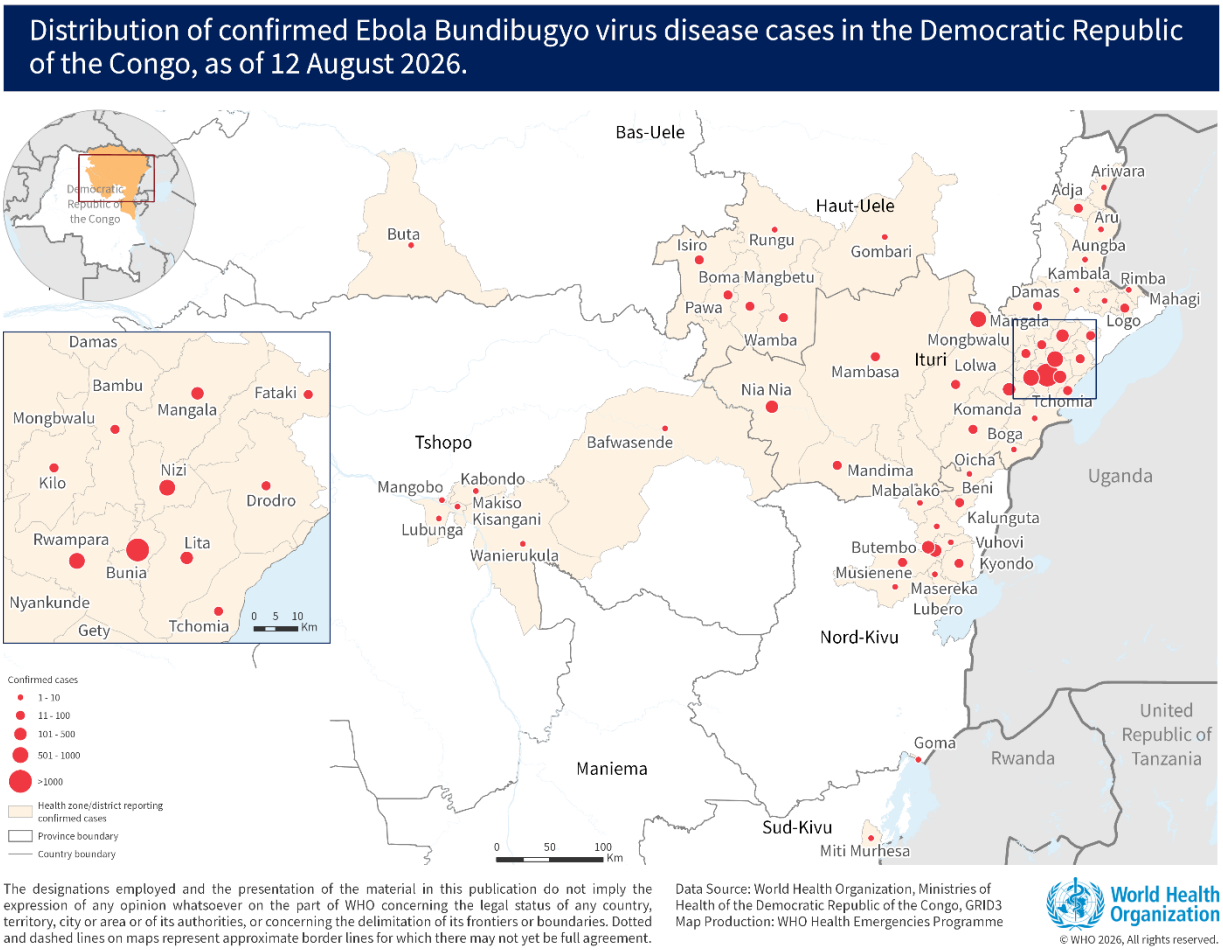
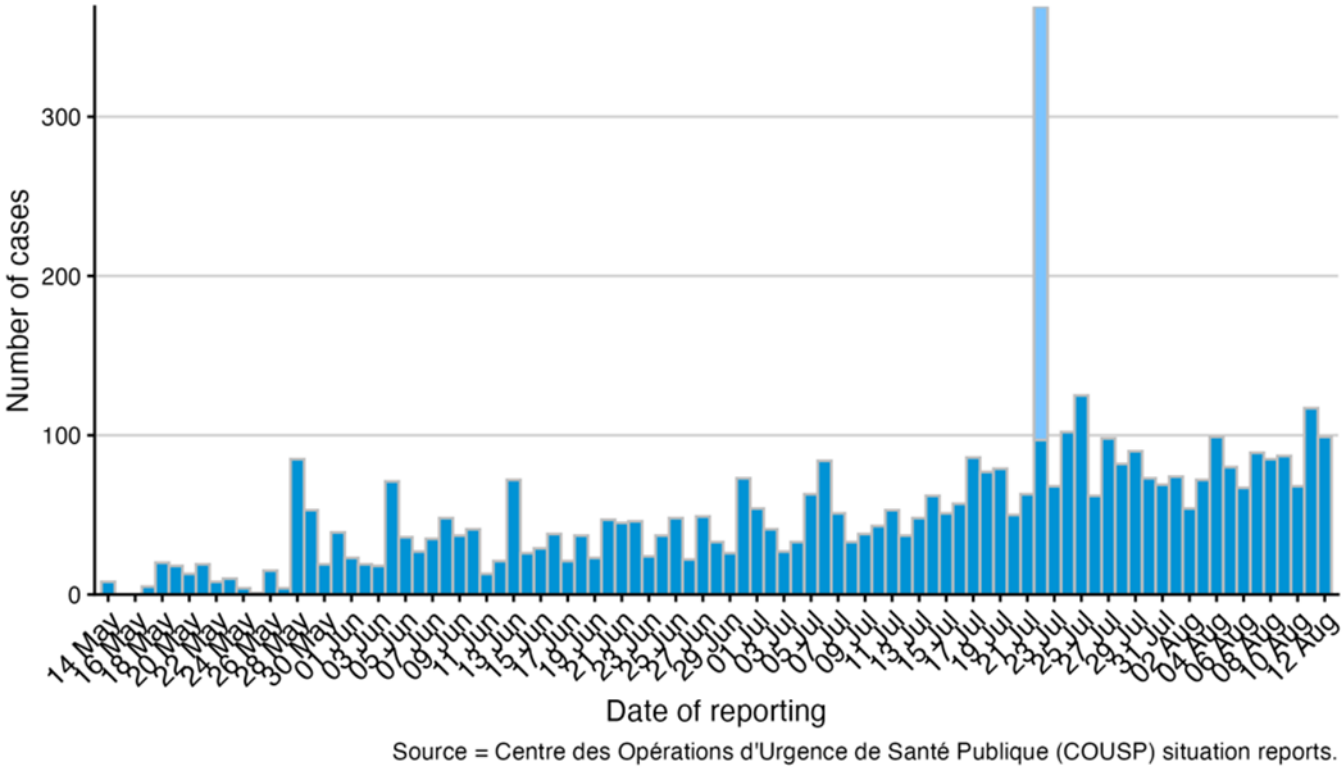


Figure 2: Epidemic curve of reported Bundibugyo virus disease cases by date of reporting, Democratic Republic of the Congo, 14 May–12 August 2026.



Source = Centre des Opérations d'Urgence de Santé Publique (COUSP) situation reports.

Figure 3: Number of confirmed Bundibugyo virus disease deaths by date of reporting, Democratic Republic of the Congo, 14 May–12 August 2026.

